

Policy on Safe Administration of Medications

«Hospital Name»

Document control

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Prepared by	Nursing Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Medications are administered only by permitted personnel, only after the patient's identity and the medication order are fully verified, with tubing connections traced to prevent misconnection, and every dose documented accurately.

1. Purpose

This policy explains how «Hospital Name» ensures only permitted personnel administer medications, verifies patient identity and the order before every dose, prevents tubing and catheter mis-connections, documents every administration, and governs self-administration and outside medications.

This policy does not cover prescription, dispensing, or storage of medications in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all staff who administer medications at «Hospital Name».

3. Policy standards

«Hospital Name» ensures medications are administered only by those permitted by law, with the patient identified using at least two identifiers and the medication, strength, route, and timing verified against the order and physically inspected before every dose. Measures

prevent catheter and tubing mis-connections. Every administration is documented in a uniform location, and where self-administration or outside medications are permitted, written guidance governs how.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Do not let anyone administer medication who isn't a registered nurse or doctor with at minimum an MBBS qualification, unless another staff category is specifically backed by legislation or a government order.
2. Do not prepare a second drug before labelling the first one, wherever more than one drug is prepared and loaded.
3. Do not administer medication without identifying the patient using at least two identifiers, one of which is the unique identification number and one of which is the patient's full name.
4. Do not administer medication without verifying it against the order and physically inspecting it — checking general appearance and expiry date — and do not proceed if any order parameter (name, strength, route, frequency, or time) is missing or incomplete; defer instead, pending verification with the treating team.
5. Do not treat a verbal confirmation of an order as anything other than a verbal order — follow the organisation's verbal-order process.
6. Do not administer a high-risk medication without independent verification by at least two staff members, documented.
7. Before administering any medication, staff shall verify the strength, route, and scheduled time individually against the written order; these three checks are distinct steps and no single verification satisfies the requirement for the others.
8. Do not use an IV extension tube for an epidural, irrigation, a drain, a central line, or to extend an enteric feeding tube, and do not make any connection or reconnection without first tracing the line from its origin to the connection port.
9. Do not document medication administration anywhere other than the uniform location, and do not batch-chart multiple doses at shift-end instead of documenting each dose separately as it happens.
10. Do not permit patient self-administration, or accept medications brought from outside the organisation, without written guidance governing it.

5. What we do

5.1 Administer medications only by permitted personnel

Only a registered nurse or a doctor with at minimum an MBBS qualification administers medication. Where another staff category is authorised to administer medication, that authorisation is backed by legislation or a government order, not an informal internal decision. «Hospital Name» keeps an authorised-personnel list.

5.2 Label medications when preparing more than one

Wherever more than one drug is prepared and loaded — for example anaesthetic drugs in the OT, or chemotherapy drugs — the first medication is labelled before the second is prepared. This applies throughout the organisation, not just in these example areas.

5.3 Identify the patient before administering

At least two identifiers are used to confirm the patient before administration — one is the unique identification number (for example the hospital or IP number), and one is the patient's full name. Matching only by bed number, or relying on an attendant's confirmation alone, doesn't meet this requirement. (Related requirements are covered in the hospital's other policies.)

5.4 Verify and inspect the medication before administering

Staff verify the medication order and physically inspect the medication — checking its general appearance (for example, melting or clumping) and expiry date — before administration. Where any order parameter (name, strength, route, frequency, or time) is missing or incomplete, administration is deferred pending early verification by the treating team. Where verbal confirmation of an order is obtained, it is treated as a verbal order and «Hospital Name»'s verbal-order process applies. For high-risk medications, verification is done independently by at least two staff members (nurse-nurse or nurse-doctor) and documented; nurses are knowledgeable about high-risk medications and are empowered to highlight prescription errors identified during order verification.

5.5 Verify strength before administering

The strength of the medication is verified from the order before administration. Where a discrepancy is found, administration is deferred rather than adjusted at the bedside. (Related requirements are covered in the hospital's other policies.)

5.6 Verify route before administering

Where applicable, the site or route of administration is verified from the order before administration. (Related requirements are covered in the hospital's other policies.)

5.7 Verify timing before administering

Where a medication order doesn't state an explicit administration time — for example, standard frequency codes like once-daily-three-times or twice-daily — «Hospital Name» maintains documentation supporting the correct timing, and these suggested timings are followed. The ISMP Acute Care Guidelines for Timely Administration of Scheduled Medications, which classify medications as time-critical or non-time-critical, are a useful reference «Hospital Name» could adopt or adapt.

5.8 Prevent catheter and tubing mis-connections

«Hospital Name» ensures inadvertent administration of a drug through the wrong route is avoided. IV extension tubes are never used for epidurals, irrigation, drains, or central lines, and never used to extend enteric feeding tubes. Functionally dissimilar tubes used in

patient care are positioned away from one another. Staff trace every line from its origin to the connection port to verify the correct attachment before making any connection or reconnection, or before administering any medication, solution, or other product.

5.9 Document every administration

Documentation of medication administration happens in a uniform location. It includes the medication's name, strength, route, timing, and the name (or employee ID) and signature of the person who administered it. Each dose is documented separately — not batched together at shift-end. Documentation reflects what was actually administered, not what was originally ordered — for example, if a different brand of the same generic drug was given, or half a 500 mg tablet was given in place of a 250 mg tablet, the record shows what was actually given. For infusions, the start time, rate or volume, and end time are captured; for continuous infusions, the drop rate or volume is documented and the total volume infused calculated for each shift.

5.10 Govern patient self-administration

«Hospital Name» decides, and documents, whether it permits patients to self-administer medications — for example, self-administration of insulin. Where permitted, written guidance defines which medications a patient may self-administer, it's preferable to also have a method for assisting self-administration, and the patient is reminded to take each dose, with reminders documented.

5.11 Govern medications brought from outside

«Hospital Name» decides whether it permits patients to use medications they've brought from outside the organisation, and implements the necessary measures either way. Where permitted, written guidance sets out the pre-requisites such medications must meet — for example, a clear label showing name, strength, expiry date, and batch number.

6. Stop-work authority

Do not administer a medication when the patient has not been identified with the organisation's identifiers, when the medication, strength, route, or timing has not been verified against the order, when the product fails physical inspection, or when you are not on the list of persons permitted by law and by «Hospital Name» to administer that medication.

Do not connect a catheter or tubing for medication administration until the line has been traced from the patient to the source.

Stop-work applies to the administration start. Immediate life-saving administration in a crash continues with the best available permitted staff and is documented afterward.

The person who stops tells the Nurse In-Charge and the treating doctor the same shift. Refusing an unsafe administration is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring safe medication administration is resourced and implemented; maintains the authorised-administration-personnel list together with the Nursing Superintendent.
Nursing Superintendent	Owens day-to-day implementation of this policy; maintains the authorised-administration-personnel list; ensures identification, verification, documentation, and line-tracing practices are followed; receives stop-work escalations.
Medication Safety Officer	Coordinates medication-safety processes; brings administration incidents and audit findings to the Drug and Therapeutics Committee.
Treating doctors	Provide clear, complete medication orders; countersign verbal orders within 24 hours; respond to stop-work escalations from administering staff.
Pharmacy In-Charge	Ensures medications are correctly labelled and verified before reaching the administering nurse or doctor; supports the high-risk medication verification process.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

8. Quality monitoring

Monitoring area	What is monitored
Authorised administration personnel	Only registered nurses, MBBS-minimum doctors, or legislation-backed staff administer medications; authorised-personnel list current.
Medication preparation labelling	First medication labelled before a second is prepared wherever more than one drug is prepared and loaded; applies throughout the organisation.
Patient identification	At least two identifiers — unique identification number plus full name — confirmed before every administration; bed number or attendant alone not accepted.
Medication verification and physical inspection	Order verified (name, strength, route, frequency, time) and medication physically inspected (appearance and expiry) before

	every dose; administration deferred when any parameter is missing or incomplete; two-staff independent documented check for high-risk medications.
Strength, route, and timing verification	Strength, route (where applicable), and timing verified from the order before every administration; discrepancies result in deferral.
Catheter and tubing mis-connection prevention	IV extension tubes never used for epidurals, irrigation, drains, central lines, or enteric feeding tubes; line traced before every connection or reconnection; training records current.
Administration documentation	Documentation in a uniform location for every dose — name, strength, route, timing, administrator identity; each dose documented separately, not batched; actual administration reflected, not original order alone where different; infusion records complete.
Patient self-administration	Decision documented; written guidance in place where permitted; patient reminders documented per dose.
Outside medications	Decision documented; written pre-requisites guidance in place where permitted.
Stop-work events	Stop-work events logged with trigger, action taken, and outcome.

9. Training and staff acknowledgement

All staff who administer medications shall be familiar with the authorised-personnel requirements, patient identification, medication verification and inspection, line-tracing and IV-extension-tube prohibition, administration documentation, and stop-work authority in this policy.

I have read the Policy on Safe Administration of Medications of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all staff who administer medications, the Nursing Superintendent, treating doctors, pharmacy staff, the Medication Safety Officer, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
ADR	Adverse drug reaction
CORE	Core objective element (NABH)
ICU	Intensive Care Unit
ISMP	Institute for Safe Medication Practices
IV	Intravenous
MBBS	Bachelor of Medicine, Bachelor of Surgery
MOM	Management of Medication (NABH Hospitals chapter)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OT	Operation Theatre

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.7.a	Commitment	Sections 3 and 5.1 address the registered-nurse or MBBS-minimum-doctor requirement and the legislation-or-government-order basis for any other authorised category. This is also a stop-work trigger in Section 6.
MOM.7.b	Commitment	Section 5.2 addresses the labelling requirement before preparing a second drug, wherever more than one drug is prepared and loaded.
MOM.7.c	Commitment	Sections 3 and 5.3 address two-identifier patient identification — unique identification number and full name — before every administration; bed-

		number matching alone is explicitly excluded. This is also a stop-work trigger in Section 6.
MOM.7.d	CORE	Sections 3 and 5.4 address medication-order verification and physical inspection before administration, the five-parameter completeness check (name, strength, route, frequency, time) with deferral — not adjustment — where any parameter is missing, and two-staff independent documented verification for high-risk medications. This is also a stop-work trigger in Section 6.
MOM.7.e	Commitment	Section 5.5 addresses strength verification from the order before administration, and deferral — not bedside adjustment — where a discrepancy is found.
MOM.7.f	Commitment	Section 5.6 addresses route (and site, where applicable) verification from the order before administration.
MOM.7.g	Commitment	Section 5.7 addresses timing verification — including documentation supporting the correct time where the order uses a frequency code only.
MOM.7.h	CORE*	Sections 3 and 5.8 address the absolute prohibition on using IV extension tubes for epidurals, irrigation, drains, central lines, or enteric feeding tubes, the line-trace requirement before every connection or reconnection, and positioning of dissimilar tubes away from each other. This is also a stop-work trigger in Section

		6.
MOM.7.i	Commitment	Section 5.9 addresses documentation of every administration in a uniform location, each dose separately (not batched), reflecting actual administration, with full infusion detail.
MOM.7.j	Commitment*	Section 5.10 addresses the decision on whether patient self-administration is permitted, written guidance where it is, and documented dose reminders.
MOM.7.k	Commitment*	Section 5.11 addresses the decision on whether outside medications are permitted, and written pre-requisites guidance where they are.

13. Required Records/Evidence Checklist

Authorised administration personnel — MOM.7.a

11. Authorised-administration-personnel list held by the Nursing Superintendent and Medical Superintendent, naming registered nurses, MBBS-minimum doctors, and any other category with its legislative or government-order backing.
12. Documented supervision records where student staff administer under supervision.
13. Confirmation no unlisted person administered a medication.

Medication preparation labelling — MOM.7.b

14. Observation or audit records confirming the first medication is labelled before a second drug is prepared, across OT, ICU, emergency, and wards.
15. Training records on the labelling rule for areas where more than one drug is prepared and loaded.

Patient identification — MOM.7.c

16. Administration records showing at least two identifiers used — unique identification number and full name — before each administration.
17. Training records confirming bed-number-only matching is not accepted.
18. Records of administrations stopped where identity could not be confirmed.

Medication verification and physical inspection — MOM.7.d (CORE)

19. Pre-administration verification records showing medication matched against order and physically inspected for appearance and expiry.

20. Deferral records where any order parameter (name, strength, route, frequency, or time) was missing or incomplete, pending treating-team verification.
21. Two-staff independent documented verification records for high-risk medications before administration.
22. Training records confirming nurses are trained on high-risk medications and empowered to raise concerns.

Strength, route, and timing verification — MOM.7.e, MOM.7.f, MOM.7.g

23. Strength verification records from order before administration; deferral records for discrepancies.
24. Route verification records (where applicable) before administration.
25. Timing documentation showing correct administration time where the order uses a frequency code only.

Catheter and tubing mis-connection prevention — MOM.7.h (CORE*)

26. Written measures in place describing the IV-extension-tube prohibition (epidurals, irrigation, drains, central lines, enteric feeding tubes) and the line-trace requirement before every connection or reconnection.
27. Line-tracing records or observation evidence before connections for IV, epidural, and enteral medication administration.
28. Training records for ICU, OT, emergency, and ward staff on catheter and tubing mis-connection prevention.

Administration documentation — MOM.7.i

29. Medication chart or equivalent uniform-location records showing name, strength, route, timing, and administrator identity for each dose.
30. Confirmation each dose is documented separately — no shift-end batch-charting.
31. Records reflecting actual administration where it differed from the original order (brand substitution, half-tablet, infusion rate or volume adjustment).
32. Infusion records showing start time, rate or volume, and end time; continuous-infusion records showing drop rate and total volume per shift.

Patient self-administration — MOM.7.j (Commitment*)

33. Documented organisational decision on whether patient self-administration is permitted.
34. Written guidance defining which medications a patient may self-administer, where permitted.
35. Dose-reminder documentation records, where self-administration is permitted.

Outside medications — MOM.7.k (Commitment*)

36. Documented organisational decision on whether patients may bring their own medications.
37. Written pre-requisites guidance (for example, clear label showing name, strength, expiry date, and batch number), where outside medications are permitted.

38. Records showing outside medications identified by pharmacy or the treating doctor before use.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.7.
2. Institute for Safe Medication Practices (ISMP)/ECRI. Acute Care Guidelines for Timely Administration of Scheduled Medications. ISMP; 2011 (and subsequent revisions). ISMP's resources are now maintained by ECRI at: <https://home.ecri.org/blogs/ismpr-resources>

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

Several requirements in this document are statutory rather than advisory — in particular those arising under applicable laws governing who may administer medications as named in NABH MOM.7.a. Statutory requirements change, and State authorities impose additional or stricter conditions. «Hospital Name» is responsible for verifying the current text of any rule cited here and the conditions attached to its own authorisations and licences; this document does not constitute legal advice.

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